

UNDERSTANDING ALZHEIMER'S

**A GUIDE FOR FAMILIES, FRIENDS,
AND HEALTH CARE PROVIDERS**



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This guide is dedicated to all the families, friends, and health care workers who lovingly support those with Alzheimer's disease.

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ALZHEIMER'S DISEASE AND RELATED DISORDERS



The chance of an elderly person becoming confused, forgetful, and developing severe memory problems increases with each passing year. Alzheimer's disease, one of the most common forms of dementia, is thought to affect 5 percent of people over age 65 and 20 percent of people over 80.



What is dementia?

Dementia is impairment in brain functioning resulting in problems with memory and judgment. It is often accompanied by confusion.

In all dementias there is a loss of intellectual abilities. This means losing the ability to use information once known or learned, as well as basic abilities to think and understand. There are also memory deficits or losses with recent memory being the first area affected.

Memory loss and loss of intellectual abilities can result in:



- ▶ a person no longer being aware of social niceties, such as how to behave in public
- ▶ confusion and disorientation
- ▶ changes in abstract thinking (such as being unable to understand humor or discern the passage of time)
- ▶ severe problems in what is called "higher order functioning" (such as language use and the ability to do numerical calculations)
- ▶ a change or exaggeration in the personality of the individual, for example, becoming more stubborn or suspicious
- ▶ problems with personal hygiene

What is Alzheimer's disease?

Alzheimer's disease is a progressive, irreversible brain disease that affects the central nervous system. Alzheimer's disease, which is the

most common form of dementia, is not part of normal aging. It is caused by factors that are largely unknown and result in a slow, gradual decline of intellectual functioning, memory, and judgment. It can occur as early as 40 years of age or as late as 100 years of age.

What Alzheimer's disease is NOT

- ▶ Alzheimer's disease is not contagious
- ▶ Alzheimer's disease is not insanity or "craziness"
- ▶ Alzheimer's disease is not an inevitable result of aging



What is physically happening in Alzheimer's disease?

Several major changes occur in the brain:

- ▶ the nerve cells important in memory and cognition are dying
- ▶ microscopic changes called plaques and tangles are developing
- ▶ there is a loss of certain brain chemicals, in particular, acetylcholine, which is needed for communication between nerve cells

What are the causes?

Alzheimer's disease affects men and women of all races and socioeconomic backgrounds. The cause is currently unknown. Possible causes may include:

- ▶ **Genetic factors:** In families where Alzheimer's occurs in several generations, genetic causes are suspected. In a very small number of these families, gene mutations have been found. For the large majority of families with a history of Alzheimer's disease, however, the genetic causes have yet to be determined. Genetic causes appear to be more likely in families where there is an early onset of the disease (before age 65).
- ▶ **Non-genetic factors:** These include environmental toxins such as exposure to aluminum, infectious or transmittable agents,

and traumas (such as severe head injuries). Research to date is inconclusive as to the effects of these conditions.

- ▶ ***A combination of genes, the environment, and the aging process:*** Most cases of Alzheimer's disease are thought to be a result of these three factors.

Is there a cure?



Some forms of dementia do have cures. Alzheimer's disease does not. However, there is both medical and non-medical treatment available to help manage symptoms.

- ▶ ***Medical treatment*** can include medications that may help improve symptoms.
- ▶ ***Non-medical treatment*** may help the person adjust and cope with the disease. It may also help caregivers manage the sometimes bizarre and difficult behavior that those persons may exhibit.

Over the years there have been a number of reports of "miracle" cures for Alzheimer's disease. None of them have been scientifically proven to work. People with dementia and caregivers should be wary of expensive treatments that claim to be cures.

How long will the disease last?

The course of Alzheimer's varies tremendously. The life expectancy for people with the disease can range anywhere from 3 to 25 years. It appears, however, that the average duration of the disease may be 8 to 10 years.

Is a diagnosis of Alzheimer's disease usually accurate?

A definitive diagnosis can only be made by examining the brain tissue after the person's death. However, if the person is given a thorough assessment (see pages 5-6) the diagnosis can be almost 90 percent accurate.

Are there other kinds of dementia besides Alzheimer's?

Although Alzheimer's disease is the most common form of dementia, there are other progressive dementing illnesses. These include the following:

- ▶ Vascular dementia (also known as multi-infarct dementia) is the second most common form of dementia; individuals often experience sudden deterioration, which may progress over time; sometimes only very specific areas of functioning are affected, such as speech
- ▶ Frontotemporal dementia (also known as Pick's disease), typically causes impulsive behavior, difficulty in thinking and problem solving, and language disturbances
- ▶ Dementia with Lewy Bodies, a slowly progressive disorder that may cause attention disturbances, poor judgment, sleep problems, and visual disturbances
- ▶ Parkinson's disease, in which individuals develop severe problems of movement and balance, and other problems, including dementia
- ▶ Huntington's disease, a hereditary disease usually appearing when the individual approaches 40 years of age, passed on from parent to child, whose symptoms include quick, jerky movements of the face, limbs, and trunk, and eventually dementia
- ▶ Dementias associated with physical disorders such as diabetes, thyroid disease, brain tumors, or Acquired Immune Deficiency Syndrome (AIDS)
- ▶ Dementias of alcohol or substance abuse; these may be a combination of direct chemical damage to the brain combined with nutritional or vitamin deficiencies

Are some dementias reversible?

Certain dementias can be reversed or improved. These include dementias associated with metabolic disorders such as diabetes, or dementias associated with alcohol or substance abuse.

Also, there are a number of physical problems which can have dementia-like secondary symptoms. These include vitamin deficiencies, reactions

to medications, physical illnesses, delirium, anemia, and hearing or vision problems. Often, once the physical problem is treated, the dementia-like symptoms will subside.

Two treatable conditions commonly confused with dementia are delirium and depression. These two conditions will be discussed on pages 8-9.

What are the tests needed to diagnose Alzheimer's?

Assessment may consist of:

► 1. *A physical examination*

- a comprehensive physical examination to find any treatable medical conditions such as high blood pressure or anemia
- laboratory studies including a blood count, urinalysis and blood chemistry screening; assessment of liver, kidney and thyroid functioning
- X-rays of the head, such as a CAT scan
- hearing and vision exams



► 2. *Family interviews*

Families or others knowledgeable about the person must be interviewed to obtain accurate information about past and current problems including:

- drug and alcohol use
- medication use
- a history and progression of cognitive and behavioral problems
- a medical and family history

► 3. *Mental status (neuropsychological) tests*

- cognitive tests to determine whether the individual is actually having memory problems and how severe these problems are
- a psychological exam to rule out depression or other emotional illnesses

An *environmental assessment* may also be included as part of the evaluation. This assessment determines:

- if the person's home is safe and secure enough given their current level of functioning
- if changes need to be made to allow the person to remain at home
- how well the person is able to perform “activities of daily living” (such as bathing or dressing)

What are some examples of cognitive tests?

Two commonly used cognitive tests are:

- ▶ the Mini Mental State Examination (MMSE) (Folstein, Folstein, and McHugh, 1975)
- ▶ the Blessed Dementia Rating Scale (Blessed, Tomlinson, and Roth, 1968)

These tests are screening measures that identify the existence of memory problems. They are administered by trained personnel. A number of factors can influence a person's scores, including education, anxiety, and depression. The tests cannot identify the causes of memory problems.

If these tests show that some memory impairment exists, it is important that the person undergo more detailed cognitive testing. More detailed tests can determine what areas the person is having difficulty with (such as judgment, abstraction, reasoning, orientation, attention, or calculation).

Are there stages to Alzheimer's disease?



Every person reacts differently to the disease. Some people may have a slow, gradual decline; some a rapid decline; and some periods of slow or relatively little decline interspersed with periods of rapid decline. Despite this variation among individuals, there are sets of symptoms commonly seen in the beginning, middle and end phases of the disease.

Early phase

In this phase, something appears to be wrong with the person but it is hard to pinpoint the problem. The individual may:

- ▶ have difficulty concentrating
- ▶ avoid new situations
- ▶ have difficulty understanding abstract concepts (that is, not understand humor as readily, or have difficulty understanding concepts such as the past and future)
- ▶ mix up words or be unable to recall words
- ▶ forget recent events or cover up forgetfulness (that is, give vague answers or make up answers)
- ▶ have problems with mathematical calculations
- ▶ have a personality change (act in unusual or extreme ways)
- ▶ be unable to deal with ordinary problems such as managing finances, paying bills, keeping clothes clean, tending to personal hygiene, or doing necessary household tasks

Everyone has memory lapses—not remembering a name, forgetting where you left your keys—but with Alzheimer’s disease these problems occur frequently and worsen over time.

Middle phase

Impairments in memory and functioning become more obvious in this phase. Although memory from the distant past is often still clear (such as remembering stories from young adulthood), a person may forget more recent events such as what they did earlier in the day or yesterday. Sometimes they won’t recognize friends or neighbors. They may also:

- ▶ experience worsening judgment
- ▶ need help making decisions, performing tasks, or planning for the future
- ▶ become less sociable and less aware of feelings of others
- ▶ have difficulty sleeping at night

- ▶ have difficulty using language or remembering common words
- ▶ get lost when visiting unfamiliar places
- ▶ need assistance with bathing or complex tasks
- ▶ experience personality changes (such as sudden mood shifts, outbursts of anger, extreme worry or fearfulness, or crying spells)

Advanced phase

The person's abilities have declined dramatically by the final phase. They may:

- ▶ become unable to use language
- ▶ get lost easily
- ▶ be incontinent of urine or stool
- ▶ need assistance with simple tasks
- ▶ show minimal emotional response
- ▶ walk with a shuffle
- ▶ fail to recognize familiar objects and people
- ▶ at the very end phases, the person is unable to eat or walk without assistance



What is delirium?

Because many of the symptoms of delirium are similar to Alzheimer's disease, the two are commonly confused. Delirium, however, is a **treatable** condition which can be reversed when the problems causing it are corrected. In delirium there is often a rapid onset of symptoms related to a sudden change in the person's health. Common delirium symptoms include:

- ▶ fluctuation in consciousness
- ▶ visual or auditory hallucinations
- ▶ sudden onset of incoherent speech
- ▶ sleep-wake cycle reversals (awake at night and very sleepy during the day)

Some possible causes of delirium include:

- ▶ medications—new dosage, sudden sensitivity or reaction with another medication
- ▶ vitamin deficiencies associated with chronic alcohol intake
- ▶ physical illness
- ▶ surgery and anesthesia—effects of surgery and anesthesia on the person's system may produce delirium in some people as they are recovering
- ▶ location or living changes

Dementia and delirium can occur together when a demented person:

- ▶ has a medical condition which becomes unstable (their diabetes gets out of control)
- ▶ has surgery
- ▶ is hospitalized or relocated

When these things happen, treating the cause of the delirium will greatly improve the person's functioning.

What are the signs of depression?

Like delirium, depression is a treatable condition that is commonly confused with dementia (especially the early stages of dementia). Because medication and psychotherapy can have a very positive outcome on depression, it is important not to let depression go untreated. Symptoms of depression include:

- ▶ loss of interest and pleasure in usual activities
- ▶ appetite change
- ▶ sleep disturbance (too much or too little)
- ▶ fatigue and inactivity
- ▶ trouble concentrating
- ▶ feelings of sadness



- ▶ feelings of worthlessness or guilt
- ▶ thoughts of death or suicide
- ▶ feeling discouraged about the future

Depression may be brought on by an upsetting experience or loss, or a change in the person's nervous system that is beyond his or her control.

Remember that depression and dementia can co-exist:

- ▶ one-third of individuals with early stage dementia are depressed
- ▶ depression is common after a stroke

Being able to recognize demented, delirious, and depressed behaviors and to obtain the necessary care for each is an important and useful skill for caregivers. Taking care of treatable disorders makes caregiving easier.

Are there any medications that can help with Alzheimer's symptoms?

Currently, no cure for Alzheimer's disease exists. All medication treatment for Alzheimer's disease is supportive, not curative. New medications appear with some regularity and time can change what medications are, and are not used. When considering or continuing a medication, a close relationship with your physician is critical. Only he or she can suggest what is best and monitor the person adequately.



BEHAVIOR MANAGEMENT TECHNIQUES

People who have Alzheimer's disease often pose behavior management problems for those who provide care. Since Alzheimer's disease and other dementias attack the brain in different places and at different rates of speed, you will find that each person's behavior is different. But overall, when caring for persons with dementia, you will see certain types of behavior problems. You can learn techniques to change behaviors that may be making your job difficult. Most importantly though, these techniques will help you observe and manage people's behaviors as they change over time and will allow you to adapt and change your interactions for the best results.

Demented persons show a variety of different behavioral problems, including (but not limited to) anger, agitation, depression, suspiciousness, paranoia, wandering, sexual inappropriateness, hallucinations, and delusions. All of these behaviors can pose serious difficulties for the person trying to provide care.

What is meant by the term “behavior”? Behavior is an action that you can see and describe. While a behavior might result from an internal emotion, this section will not be exclusively concerned with emotions, but will concentrate on behavior, or on what you see happening. Because you can see behavior, you can get information about it such as how often does it happen, or, who is around the person when it happens?

Why work with behaviors?

Although you can’t yet change the course of the person’s disease or how it will damage the brain, you can change behaviors. Behavior affects the quality of both of your lives. By changing an unpleasant behavior you can increase the quality of life for the person and make caregiving easier on yourself.

Changing behaviors involves the following steps:

1. Learning to observe and define problem behaviors
 2. Developing a plan to change the behavior
 3. Evaluating your plan’s effectiveness in changing behaviors
 4. Changing your plan and reevaluating to make your plan more effective
- Before starting to work with difficult behaviors, be sure that the person’s medications or medical conditions are not causing or contributing to their problems. Be alert for sudden illness, effects of new medication or change in dosage, or any other recent changes in their life or surroundings.

Defining a “problem” behavior

A behavior can be considered a “problem” if for some reason it is not suitable or acceptable to you or others. This could mean that the

behavior is dangerous to someone (like striking someone), or it damages something (like breaking objects), or it is unpleasant to experience (like yelling or arguing). Sometimes several behavior problems occur at once (the person yells and strikes out).

Observing behavior

Behaviors always occur in three parts. These parts we call **A, B, and C**.

- ▶ **A** is a triggering event (often called an activator or cause)
- ▶ **B** is the behavior itself
- ▶ **C** is the consequence of the behavior (what happens because of the behavior)

As you closely observe a behavior you will learn to look for these three parts and they will be your key to changing the behavior. Think about the problem and gather information:

- ▶ *First, define the **behavior (B)**.*

What is the current behavior? What is happening? What did the person do? Describe the action—for example, the person yelled, the person struck out, the person paced, the person asked the same question ten times, or the person pinched the nurse's aide. Also notice when, where, and how often behavior occurs.

- ▶ *Next, look for the prior events that may have triggered the behavior (the **activator (A)**).*

Did anyone or anything trigger this behavior? What was happening before the behavior started? Look for clues, like somebody demanding too much, being angry at the person, or for disturbing noise or activity around the person. Were there any changes in the environment beforehand?

- ▶ *Lastly what is the **consequence (C)** of the behavior?*

What happens as a result of the behavior? What changes occurred in the environment or in the behavior of other people because of this person's behavior?

Observe and write down your observations about the behavior. It's easiest to work with one behavior at a time, so target just one to watch. Behavior, even in a confused individual, results from a cause. Understanding the causes can help you figure out what to do to help.

Developing a plan

Once you have identified a behavior, gathered information about it, and viewed the patterns of its occurrence, it is time to see if you can change it by developing an individual plan. A plan that is individualized, designed specifically around the **ABC's** of a given situation, is likely to be much more effective than one developed generally for everyone.

It is important to set realistic, reachable goals tailored to the individual. Divide your plan and goals into small easy-to-do parts. Be creative.

Changing behaviors can be very hard work so don't forget to reward yourself and the person for successes, no matter how small. Also, changing behavior is an ongoing process. What was once successful may no longer work over time, so remember to continually evaluate and change your plan as necessary.

Does changing behaviors mean ignoring emotions?

Working with behaviors doesn't mean ignoring feelings and emotions. You should focus on the behavior but keep in mind that feelings are often displayed in behavior. As the person's ability to communicate and understand the world around them gradually declines, a friendly touch, a smile and a reassuring tone may be the only way to communicate to the person that they are safe and that you care about them.



SPECIFIC STRATEGIES FOR COPING WITH PHYSICAL AND BEHAVIORAL PROBLEMS

Difficulty communicating

Effective communication is an exchange of thoughts, information, and other messages from one person to another. Since persons with dementia have a brain impairment, it can be very difficult for them to

understand and communicate. Word finding and other communication problems can leave them feeling anxious, agitated, and upset. To help minimize these problems, remember to:

- ▶ allow enough time for communication—give the person plenty of time to listen and respond
- ▶ demonstrate visually—show them what you mean
- ▶ maintain good non-verbal behavior—use eye contact, touch, smiles, and move slowly
- ▶ keep at the person's eye level—if they're sitting or standing, position yourself at their level
- ▶ provide multiple clues and suggestions—"fill in the blanks" for them by making suggestions, one at a time
- ▶ change the subject—if you can't understand them after trying everything, take a break and try again later
- ▶ stay calm—the person needs you to remain calm, even under pressure
- ▶ do one task at a time

Poor vision

Poor vision can add to already existing communication difficulties for persons with dementia. Vision is an important part of communication. Watching facial expressions, establishing eye contact, seeing gestures and nods can all help good communication. Poor vision can also increase risk for falls and injury.

Poor vision may include apparent signs of clumsiness, falls, and bumping into things. To cope with such problems:

- ▶ eliminate visual distraction (such as too much furniture)
- ▶ experiment with lighting—not too dark and not too much glare
- ▶ when speaking to the person, make sure you're in front of them and they can see you; avoid sitting or standing at their side.
- ▶ decrease use of gestures and non verbal communication
- ▶ check the person's medications, side effects can include blurred vision

Hearing problems

Hearing problems can aggravate the symptoms of dementia. They can add to a person's isolation and confusion and, in some cases, are related to auditory hallucinations and paranoia. To cope with hearing problems:

- ▶ make sure the person can see you well (don't sit at their side)
- ▶ use eye contact and touch
- ▶ decrease distracting background noise (such as the radio or TV)
- ▶ speak slowly and clearly without shouting; also try lowering the tone or pitch of your voice (low tones are easier to hear)
- ▶ use gestures or demonstrations
- ▶ double-check to make sure you are understood

Pain

Persons with dementia may be unable to tell you when they are experiencing pain. It's important that you look for secondary signs such as:

- ▶ increase in agitation or irritability
- ▶ sensitivity to touch
- ▶ facial cues such as grimacing

If you notice these symptoms, or suspect the person is in pain, consult your physician.



Loss of motor skills or driving ability

Deciding whether a person should or shouldn't drive is a difficult decision. Driving is often associated with one's feelings of independence, adulthood and freedom, unfortunately, a motor vehicle is also a lethal weapon. Persons with dementia often experience problems that make driving dangerous. If they are unwilling to stop driving you may want to:

- ▶ request that the Department of Motor Vehicles test the person's driving skills
- ▶ explain the risk to your doctor and have him or her write a

prescription saying the person shouldn't drive

- ▶ disable the car
- ▶ take away the keys

Whatever you do, be sure to arrange for alternate transportation so that the person does not become isolated when they stop driving.

Wandering

Wandering is a stressful and potentially dangerous symptom of dementia. It is important to consider the possible causes of wandering, including:

- ▶ stress created by too much noise or activity
- ▶ unmet basic needs such as food, toilet, or companionship
- ▶ physical problems such as pain
- ▶ inactivity
- ▶ changes in weather
- ▶ confusion about the time of day
- ▶ searching for a familiar living environment or person
- ▶ searching for a lost object
- ▶ past routines
- ▶ a reaction to hallucinations or delusions

How to prevent wandering

Even though wandering can seem like an insurmountable problem, there are steps you can take to prevent or minimize wandering. These steps include:

- ▶ remove things that suggest leaving, such as coats or hats
- ▶ increase daily exercise
- ▶ decrease or increase the person's stimulation



- ▶ check the person's vision and hearing
- ▶ determine if there is a pattern to the person's wandering (certain time of day or a certain path)
- ▶ determine if there is a cause (needing something or searching for something)
- ▶ change medications

Whether you can prevent the person's wandering or not, it is important that you make the changes necessary to insure their safety:

- ▶ provide night lights
- ▶ install door locks or alarms
- ▶ keep a current picture of the person for reference
- ▶ have the person wear an ID bracelet or tag with contact information
- ▶ provide a safe place to wander (such as a locked, fenced yard)

Catastrophic reactions

Catastrophic reactions are extreme outbursts of emotion, most often anger or agitation. Catastrophic reactions are difficult to work with so the best strategy is to work on preventing them. Common triggers for catastrophic reactions include:

- ▶ too many demands or too many questions at one time
- ▶ too much noise and activity
- ▶ difficult tasks
- ▶ particular people
- ▶ criticism
- ▶ fatigue



You can decrease the likelihood a catastrophic reaction will occur by:

- ▶ speaking slowly and softly
- ▶ making one demand at a time
- ▶ completing each task before moving on to the next

- ▶ avoiding fatigue
- ▶ making eye contact
- ▶ taking a break and trying again later

Once a catastrophic reaction has started there are some things you can do. Try to:

- ▶ soothe and empathize with the person (be sure to avoid criticism)
- ▶ if possible, remove the person from the scene of conflict
- ▶ distract or redirect the person—offer an alternate activity the person enjoys (such as taking a coffee break)
- ▶ get help if you need it

Keep your responses kind and supportive during a crisis and also remember to give praise and pay attention to the person at non-crisis times (such as when they are cooperative and pleasant). That way you will reinforce the person's behavior when they are behaving well and make it more likely that they will behave this way more often.

Bathing

As Alzheimer's disease progresses, people need assistance in caring for themselves. They may forget the need to bathe, feel frightened of bathing, or feel uncomfortable having someone help them with such a private task. To make bathing as easy as possible try to:

- ▶ ensure privacy
- ▶ have the bath water ready beforehand
- ▶ let the person touch the water
- ▶ undress them gradually
- ▶ go slowly
- ▶ give specific verbal instructions, one at a time
- ▶ avoid showering the face
- ▶ save shampooing until the last
- ▶ avoid bath oils that make the tub slippery

- ▶ install non-skid stickers and grab bars; remember that most falls and injuries occur in the bathroom
- ▶ use gentle touch
- ▶ cover up mirrors if they disturb the person

If they won't cooperate:

- ▶ give choices such as "do you want to take a bath or shower?"
- ▶ avoid confrontation
- ▶ try again later
- ▶ try another bather
- ▶ don't persist too long, it can cause a catastrophic reaction in someone who is flatly refusing
- ▶ schedule baths around upcoming events; when the person has a reason to be clean, such as going to church or having visitors, they are often more willing to bathe
- ▶ consider providing sponge baths

Dressing

Being comfortable and nicely dressed is often important in providing a sense of well being. With clear suggestions and directions, persons with dementia are often able to do a fair amount for themselves. Encourage them to do as much as possible and be aware of eliminating frustrations, such as too many choices of clothing, or items that are difficult to fasten. When assisting someone with dressing:

- ▶ lay articles of clothing out in sequence
- ▶ pick clothes that fit easily
- ▶ use specific verbal and visual cues such as "here are your socks" while handing them the socks
- ▶ give one simple instruction at a time and wait until the person is finished before moving on
- ▶ undress only one part at a time; have the next article of clothing ready to put on

- ▶ speak gently
- ▶ keep the dressing routine as consistent as possible
- ▶ if the person wants to wear the same thing over and over, obtain duplicates

Eating

Many individuals with dementia develop problems with eating. Here are some ways to prevent such problems:

- ▶ offer one food at a time; when the person finishes with one food, offer another
- ▶ make sure the place setting and foods are varied in color (otherwise it may be difficult to see what the foods are)
- ▶ try lighter weight utensils; also try serving more finger foods such as bananas, fish fillets, whole wheat bread, or breakfast bars
- ▶ check for dental problems, poor fitting dentures, mouth dryness, or swallowing difficulties
- ▶ provide a relaxing eating area (for example, an uncluttered area with soothing music)
- ▶ provide adequate fluids (dehydration is a common problem with persons with dementia); avoid caffeine (it's a diuretic) and serve jell-o, popsicles, juices, and ice cream to increase fluids
- ▶ consider the side effects of medications (some medications may induce nausea or affect appetite)
- ▶ ask the person's doctor if a dietary supplement such as Ensure is needed to provide additional nutrition

Medication safety

As a caregiver, it is important that you understand the effects of medications and how to use them. Be alert to possible side effects and be careful to have your physician evaluate all medications regularly. Some other medication reminders are:

- ▶ don't ever change dosages without consulting your physician

- ▶ try to give medication at a specific time and in a specific way each time
- ▶ check to make sure the medication has been swallowed; crush pills and mix with applesauce if the person has trouble swallowing
- ▶ keep medications in a locked place
- ▶ watch for uncharacteristic changes in the person's mood (such as excessive tiredness or agitation); consult your physician promptly to check for overmedication

Toileting

Good toileting habits can prevent an array of health and behavioral problems. Some things to consider:

- ▶ make sure the bathroom is clearly marked with a sign (use a picture if the person no longer understands written signs)
- ▶ if the person begins having trouble with incontinence, take him or her to the bathroom every two to four hours; do this on a routine basis (such as before meals)
- ▶ continually evaluate the level of assistance needed
- ▶ provide adequate lighting along the pathway to the bathroom
- ▶ remember that urinary tract infections are common in elderly people; if a fever persists for more than 24 hours, contact their doctor

Depression

About 20 to 30 percent of people with Alzheimer's disease will also develop depression. Depression is treatable. Make sure you are aware of the symptoms and seek help when they occur. Here are some common signs of depression:

- ▶ talking about or appearing "sad" or "down"
- ▶ decreased interest in previous activities
- ▶ crying more than usual
- ▶ significant weight loss or weight gain
- ▶ eating more or less than usual

- ▶ insomnia—not being able to sleep
- ▶ hypersomnia—sleeping too much
- ▶ agitation (or restlessness)
- ▶ talking less than usual
- ▶ feelings of guilt
- ▶ lack of concentration
- ▶ fatigue
- ▶ feelings of worthlessness
- ▶ recurrent thoughts of death or suicide

If someone can no longer speak, watch their body language. See if you notice changes in the person's mood:

- ▶ at certain times of day
- ▶ around certain people
- ▶ during certain events

If depression is suspected, a physical exam is necessary to be sure the person with dementia doesn't have a medical condition that is affecting their mood. It is unknown what really causes depression. Some depression may be brought about by:

- ▶ an upsetting experience
- ▶ repeated changes
- ▶ sad incidents
- ▶ changes in the nervous system that are beyond the person's control

What can you do to reduce depression?

Emotions, behavior, and thoughts all work together to maintain depression and contribute to a downward spiral of mood. In a depressed person it is important to try to:

- ▶ break the cycle of inactivity
- ▶ increase pleasant events

Use your knowledge of the person to find activities and schedule them regularly (such as daily or weekly). Give them lots of reassurance and support for participating. Some ideas for pleasant activities include:

- ▶ listening to or singing to music
- ▶ looking at old photos
- ▶ talking about favorite topics
- ▶ going for walks or rides
- ▶ being with animals or children
- ▶ doing simple projects

For pleasant events to be successful they must:

- ▶ be fun
- ▶ be done in small steps
- ▶ be introduced slowly
- ▶ be used with encouragement
- ▶ fit the person's abilities



Paranoia or suspicious behavior

About one-third of people with Alzheimer's disease or other dementias develop paranoid or suspicious behaviors. These behaviors can be very disturbing to caregivers. It is important not to take these behaviors personally. Be aware of factors that can aggravate paranoia:

- ▶ sensory deficits (hearing, vision) can cause the person to withdraw or to misconstrue events
- ▶ boredom
- ▶ environmental problems (lighting/noise)
- ▶ sensory overload (too much noise/activity)
- ▶ changes in daily routine or the environment
- ▶ medications
- ▶ daylight changes in the late afternoon and early evening

When the behavior occurs, don't try to orient, correct, or argue with the person (this can just make them feel frustrated). Instead try to:

- ▶ redirect or distract their attention
- ▶ use physical touch when appropriate
- ▶ reassure the person that they are safe and you will take care of them

Some ways to avoid paranoid or suspicious reactions are to:

- ▶ increase lighting
- ▶ give regular reassurance
- ▶ regulate people contact
- ▶ maintain a daily routine and minimize changes
- ▶ keep them active
- ▶ avoid stressful activities in the late afternoon or early evening; make sure they are well rested and have adequate attention from you during this period

Hallucinations and delusions

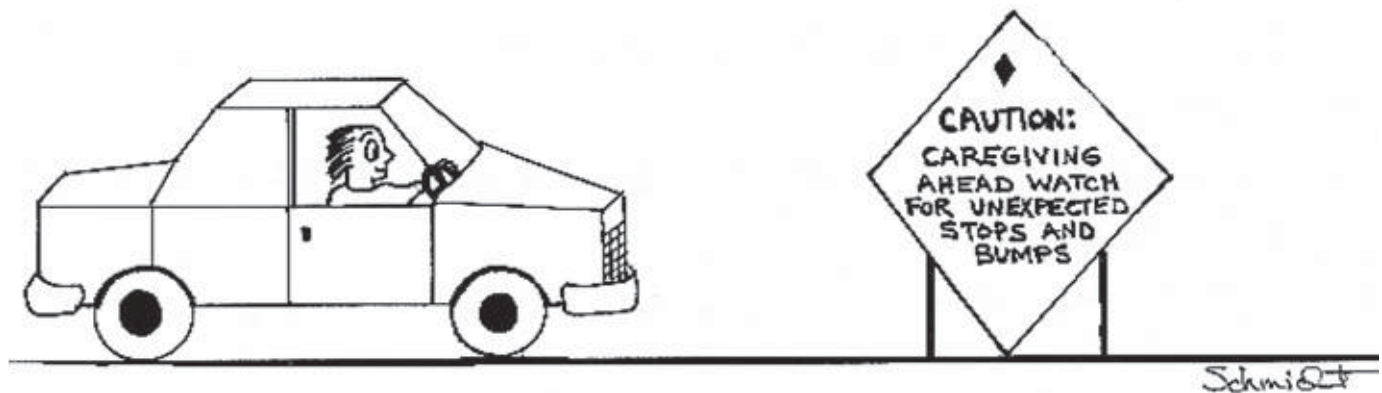
Hallucinations are seeing, hearing, smelling, tasting, or feeling things that aren't there. Hallucinations occur in about 20 percent of people with Alzheimer's disease. The most common hallucinations are visual and auditory.

Delusions are beliefs that are contrary to the truth (such as when a person thinks they are at a stranger's house when in fact they are at home).

Both hallucinations and delusions are secondary symptoms of the disease and do not mean that the person is "going crazy." It is important to remain calm, consistent, and supportive of the person when hallucinations or delusions occur. The following are some suggestions for managing these symptoms:

- ▶ don't tell the person that they are "imagining things"—this often just upsets them

- ▶ respond to the fears and the feelings being expressed by saying, “that must be scary” or “it must be difficult for you”
- ▶ let them know that you are available to them when they feel frightened
- ▶ maintain a soothing voice
- ▶ make sure you offer as much or more attention to the person during the times they are not having hallucinations or delusions
- ▶ change the subject or distract the person
- ▶ check the environment for glare, shadows, or objects which might be misconstrued (sometimes events or people on television are perceived as being real; mirrors may also be confusing to severely demented people)
- ▶ eliminate noises that might confuse them



CAREGIVER ISSUES

Caregivers, whether they are paid, unpaid, family, friends or professionals, face many pressures and stresses. To take the best care of a person with dementia, you must take good care of yourself. Some ways to do this include the following:

- ▶ avoid isolation—talk to other caregivers
- ▶ seek support from family and friends
- ▶ find organizations that can help

- ▶ attend training and informational classes to learn new skills and find out more about the disease

Many caregivers become isolated when the person requires a great deal of care. Isolation causes stress and can produce mental and physical health problems. Make sure you ask for help at all stages of the disease. Asking for help is not a sign of weakness: **it's smart caregiving.**

Impact of diagnosis on the family

Family members may have trouble accepting that their relative has dementia. Family members may also feel anxious and depressed and worry about legal and financial arrangements. It's important to remember that dementia impacts **everyone** in the family (not just the primary caregiver).

Common feelings family members and caregivers report:

- ▶ grief and sadness
- ▶ disappointment
- ▶ fear and anxiety
- ▶ embarrassment or shame
- ▶ anger
- ▶ feeling overwhelmed and alone
- ▶ discomfort about the disease

What can be done about these feelings?

Don't deny them. They are important for letting you and other family members know your areas of strength and weakness. Seek assistance for the areas of caregiving you find most difficult. Find a supportive friend or a support group to discuss feelings. If more help is needed, consult a professional (such as a psychologist, social worker, or counselor).

Some suggestions in coping with particular feelings are:

- ▶ depression—increase pleasant activities, especially social and physical activities

- ▶ grief and sadness—increase pleasant activities and talk about your feelings with others
- ▶ embarrassment and shame—talk with others who have been through a similar situation
- ▶ disappointment—try to remain flexible and accept things that didn't go the way you had hoped they would
- ▶ fear and anxiety—plan for the future, take concrete steps to face fears and alleviate anxiety
- ▶ anger—get away from the situation and try again later, find someone to provide relief care so you can have time to yourself
- ▶ guilt—talk with a friend, counselor or helping professional; guilt is particularly tricky because there is always more you can do—be realistic in deciding what constitutes doing enough
- ▶ overwhelmed—get help, try to remember that the impaired person doesn't always have to come first, plan for times when you can rest and be alone

Why involve family and friends?

Caregivers sometimes feel the need to keep the person's disease a secret from family and friends. While you must use your own judgment and do what is comfortable for you, there are some good reasons for involving family and friends right from the start of the illness and keeping them involved.

- ▶ it helps your physical and mental health
- ▶ it is good for the person to get used to other caregivers early on (it can be harder to introduce new caregivers later in the disease)
- ▶ a team approach decreases the burden of decision-making
- ▶ family involvement can decrease misperceptions and fears about the disease and help everyone cope better

How do you involve family and friends?

There are a number of ways to involve others. One, obviously, is to have private conversations to let them know what is going on. Another is to

call a meeting of family and close friends. These meetings can be good ways to share information, feelings, and solve problems. Professionals are sometimes available to help conduct these meetings. Before arranging such a meeting think through:

- ▶ what you want to accomplish
- ▶ what you need help with
- ▶ who should attend, where the meeting should be, and how long the meeting should last
- ▶ whether you need assistance in conducting the meeting—if so, ask a social worker, clergy, lawyer, or nurse to assist you
- ▶ when asking for help from family and friends remember to be specific and be direct



Caregiving changes your life and can also change your circle of friends. Replace friends who are demanding and judgmental with people who can be supportive and understanding of your situation.

What long-term plans are necessary?

Long-term planning is important to good caregiving. Caregivers must make many difficult decisions over the course of the disease. Knowing what your choices are and having support available will make these decisions easier. When making long-term plans:

- ▶ make sure you have a knowledgeable attorney to advise you about durable powers of attorney, living wills, health care preferences, or a guardianship
- ▶ make sure you have a health care team you can rely on such as a doctor who specialized in geriatrics, a nurse, and a social worker; some geriatric doctors have nurses who can make house calls and this can be invaluable in the later stages of the disease
- ▶ become familiar with assisted living, adult family homes, and nursing homes; whether or not you decide to use one, it is good to know what's

available; remember that in urban areas some residences offering specialized care for people with dementia have long waiting lists

- ▶ develop a network of family, friends, and/or community services that can help you with caregiving at all stages
- ▶ take advantage of respite (relief care) opportunities; you can find respite help through:
 - The Area Agency on Aging
 - Adult Day Health Centers
 - Family, friends, neighbors, church and community groups, or private agencies
 - The Alzheimer's Association
 - Senior Information and Assistance
- ▶ be aware of other helpful services:
 - transportation (low-cost van services, half-price taxi fares for seniors)
 - meal and chore services
 - doctors with staff who make house calls
 - mobile X-ray services
 - handyman services



To prevent isolation and stress, make sure you use as many available resources as possible.

Your health

Your physical health often depends on your ability to get help with the burden of caregiving. Being the only one the person depends on is not smart caregiving. If you develop health problems (and this happens to many caregivers), they will be without help. A common reason for nursing home placement is because the caregiver becomes ill. Some common health problems of caregivers include:

- ▶ hypertension
- ▶ substance abuse (alcohol or medications)

- ▶ insomnia
- ▶ exacerbation of preexisting illness

It's important that you take time for basic personal needs (such as exercise, proper diet, sleep, and time with friends). If your basic needs are unmet, health problems may develop.

Stress

There are a number of reasons why caring for a person with dementia is particularly stressful. In this section we will offer some suggestions for managing the stress of caregiving.

For a person with Alzheimer's disease, no matter what you do, or how well you do it, the person will decline over time. As a result you will need to:

- ▶ continually readjust your time and involvement with the person
- ▶ take time to grieve the many losses that occur during the course of the disease
- ▶ avoid taking the person's decline personally (it does not reflect on your caregiving)
- ▶ remember it's OK to be angry at your situation: it's nothing you or the person with dementia would have chosen

Caregivers must adjust to role changes in addition to the extra tasks of caregiving. For example, an adult child often becomes a caretaker for the mother or father and a spouse assumes extra burdens and tasks in the relationship. Keep in mind that:

- ▶ role changes are tremendously stressful for both the caregiver and the person with dementia
- ▶ it's normal to feel resentful and overwhelmed when forced into a new role

Caregiving is a job without clearly defined goals. There is often no limit to how much you can be doing (and to how guilty you can feel about not doing enough). It is helpful to:

- ▶ make a list of what defines doing a good job; be specific and think about the person's physical comfort and safety; when you start to feel inadequate think back to your list—are you expecting too much of yourself?
- ▶ talk to other caregivers about what you do; often someone in the same situation can understand and help you gain a realistic perspective on your situation

Caregiving will often reactivate long-standing interpersonal problems you may have had with the person. Generally, the more troubled your relationship was with the person before the disease, the more difficulty you will experience in the caregiver role. Remember, you cannot change your history, you can only work on the present.

Different stages of the disease may create different stresses. In the early stages the predominant stress is often depression over the diagnosis of dementia or Alzheimer's and changes in the person's personality, while in the later stages the predominant stress is usually the physical exhaustion of caregiving.

Caregiving adds unexpected responsibilities. Don't expect to accomplish what you did before caregiving. Think about decreasing some of your expectations (letting the house go a few extra days without cleaning, getting take-out meals, or not sending Christmas cards this year).

Caregivers may have made promises a long time ago, when the person was healthy, that they would never send them to a nursing home. Today's reality may be quite different and these promises need to be reconsidered. Assisted living and nursing homes can provide many positive services (varied staff, activities, and continual medical monitoring) that you might not be able to provide at home. Remember, choosing a residential care facility does not mean you have failed.

Some other caregiving tips

Take time outs—a short break when you feel you're getting irritated.

Don't take the person's behavior personally. Anger and stubbornness are common secondary symptoms of dementia. Try to respond in a matter-of-

fact way and remember that the person's behavior is not deliberate.

Don't assume the person understands you. Often they pretend to understand. Observe the person's actions—what they do rather than what they say—to determine if they understand.

Don't forget to laugh, do things that are fun, and be creative with problem solving. When one woman with early stage Alzheimer's began misplacing things, she and her husband created an imaginary houseguest named "George." When something was lost they would say, "Oh, that George, he's moving things around again!"

What can they still do well? Try to concentrate on their strengths rather than the abilities that have been lost.

Listen to how you sound. Does your voice sound too loud? If so, they could mistake this for anger. Are you tense? What are your facial expressions? Do your words and how you're saying them reflect how you feel and what you mean?

Maintain a consistent daily routine. Repetition and minimal changes will help to maximize their memory.

Provide lots of cues. Demonstrate and tell them how to do a task. Break tasks into small step-by-step units.

Don't try to reason or argue with the person. Because Alzheimer's affects judgment and reasoning, getting into a logical argument about "what is right" will only frustrate both of you. Instead, when the person is uncooperative or disagreeable, try changing the subject or suggest an activity.

Practice acceptance. The disease causes memory and cognitive changes, but the person you remember is still there. Take time to cherish small moments of joy.

Support groups

The Alzheimer's Association sponsors a wide range of support groups for caregivers. Different groups have different focuses so make sure you talk to your local Alzheimer's Association about the best group for your needs.

Ask your local senior center if they offer lectures or classes on caregiver issues or aging concerns.

Try individual counseling if you are not interested in groups. It is important to have the support of someone who can listen to your concerns. Geriatric clinics in local hospitals, psychiatrists, psychologists, social workers, or mental health counselors can provide individual counseling.

Are there any resources on caregiving?

There are a number of good resources available and more are published each year. Check with your local bookstore or Alzheimer's Association. There are also many resources on the Web. Some useful websites are:

Alzheimer's Association:

www.alz.org

Alzheimer's Disease Education and Referral Center:

www.alzheimers.org

National Institutes of Health: National Institute on Aging:

www.nia.nih.gov

Conclusion

We hope this manual was helpful.

Please continue to seek help and information on both the local and national level.

New research involving persons with Alzheimer's disease is often underway. Your local Alzheimer's Association or university can tell you what projects are available in your area.

Above all, don't lose hope and don't forget—
caregiving is the most humane work of all.

Additional copies can be purchased by contacting:

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